

FREE GUIDE · FOR BEHAVIOR ANALYSTS

Before you take a remote BCBA role

Twelve questions to ask in the interview — and what the answers actually tell you.

A note on why this exists.

Remote behavior-analytic work has stopped being a pandemic workaround. Roughly 42% of BCBAs now deliver some services by telehealth, and about 18% work primarily remotely. As of January 2026, all ABA CPT codes sit on the permanent CMS telehealth list.

So the question is no longer whether remote roles exist. It's whether the specific one in front of you is set up to let you practice well — or set up to make you the person holding the liability when it doesn't work.

I've spent five years as a clinical supervisor in telehealth. These are the twelve questions I'd ask, and the reasoning behind each. Read the "listen for" line as carefully as the question itself. The answer matters less than how readily it comes.

PART ONE

Supervision and caseload

01

How many technicians will I be supervising, and how many hours of direct supervision does each receive per week?

Remote supervision takes more time per person, not less. You lose everything you'd normally pick up incidentally — the hallway debrief, the glance across the room, the sense that someone's having a hard day. A caseload that was manageable in person is not automatically manageable remotely.

Listen for: a specific number, given without hesitation. Vagueness here ("it varies," "as needed") usually means supervision is whatever's left after billable hours are covered.

02

What percentage of my supervision hours can be delivered by telehealth under the payers you bill?

This varies by state and payer, and it moves. North Carolina's revised Medicaid policy sets a telehealth supervision limit for CPT 97155 at 50%. Virginia requires initial ABA assessments to be conducted in person, and non-compliant assessments aren't reimbursable. If a role is advertised as fully remote in a state with an in-person requirement, someone is going to have to cover that gap.

Listen for: whether they know the answer at all. An employer who can't answer this has not thought carefully about compliance, and you will be the one signing the notes.

03

If I'm remote and a technician is in the home, who responds when something escalates?

This is the question I'd never skip. In person, you can step in. Remotely, you can't — and a technician alone in a home with an escalating client needs to know exactly who they call and what they're authorised to do before it happens, not during.

Listen for: an actual protocol, ideally a written one. "You'd just call me" is not a protocol; it's a hope.

04

How do you assess technician competency remotely?

Competency assessment through a screen is genuinely harder. Camera angles hide things. Sessions get performed for the observer. If an organisation has moved to remote supervision without rethinking how they check skills, they've usually just kept the in-person checklist and lost half its validity.

Listen for: evidence they've adapted the method — recorded sessions, multiple camera positions, structured probes. Not just "we do the same competency checks over Zoom."

PART TWO

Licensure, billing, and liability

05

Which states will my clients be located in, and am I licensed in each of them?

Your practice location is generally the client's location, not yours. That means licensure in every state you serve. Some states also restrict enrolment — North Carolina currently bars new out-of-state BCBA's from enrolling as Medicaid providers, even while allowing existing cross-state arrangements to continue.

Listen for: who pays for the additional licences and how long they'll wait for you to obtain them. If the answer is "we'll figure it out," you're the one figuring it out.

06

Who is responsible for verifying that telehealth delivery is covered for each client's specific plan?

Coverage is not uniform. Federal Medicare flexibilities are extended through the end of 2027, but state Medicaid and commercial plans set their own rules, and those rules change mid-year. If nobody owns verification, it defaults to the clinician whose credential is on the authorisation.

Listen for: a named role or department. Billing, intake, clinical — someone specific.

07

What happens to my caseload if a payer changes its telehealth policy mid-year?

This is the scenario that catches remote clinicians. A policy shifts, services that were reimbursable stop being reimbursable, and suddenly there's pressure to see clients in person — from a clinician who was hired remotely and may live three states away.

Listen for: whether they've been through it before and what they did. Organisations that have weathered a policy change will tell you the story readily.

08

What malpractice coverage do you carry, and does it explicitly cover telehealth delivery across state lines?

Not all policies do. Some cover telehealth only within the state of issue. This is a five-minute question that occasionally saves a career.

Listen for: willingness to show you the policy language. Reluctance is itself an answer.

PART THREE

The daily reality

09

What platform do you use, who provides the equipment, and what happens when the family's connection fails?

Technology failure is not an edge case in telehealth — it's a weekly occurrence. The organisations that handle it well have decided in advance what counts as a cancelled session, what gets documented, and whether the family bears any cost.

Listen for: a policy that doesn't quietly transfer the loss onto you or onto the family.

10

How much of my week is billable, and is documentation time built into that?

Remote roles are often sold on flexibility and then structured around a billable target that only works if you document outside paid hours. Ask for the number. Then ask what the number was for people who left.

Listen for: a percentage rather than a feeling. Anything above roughly 85% billable usually means unpaid admin.

11

How do remote clinicians here stay connected to a team?

Isolation is the most underrated risk of remote clinical work. Not because it's unpleasant, though it is — because clinical judgment degrades without other people to check it against. You stop noticing your own drift when nobody is watching you work.

Listen for: structured, scheduled contact. Optional Slack channels are not a substitute for case consultation.

12

Can I speak with a BCBA who currently works remotely for you?

The single highest-signal question in this document. Everything else can be answered aspirationally. This one can't.

Listen for: a yes. A no, or a long delay, tells you more than any other answer here.

PART FOUR

Checking your own state

I've deliberately not included a state-by-state table in this guide. Requirements change often enough that any list would be wrong within a month, and wrong regulatory information is worse than none.

Here's the process I use instead. It takes about twenty minutes and it doesn't expire.

1. Start with the licensing board in the client's state

Not yours — the client's. Search for the behavior analyst licensure board and look specifically for telehealth or telepractice provisions. Note whether an in-person component is required for assessment.

2. Then check the payer, not the state

State law says what you may do. The payer says what it will pay for, and it's often narrower. For Medicaid, find the state's ABA or behavioral health policy bulletin. For commercial plans, ask the employer which plans they bill and request the telehealth provision in writing.

3. Ask what changed in the last twelve months

Put this question to the employer directly. Policy in this area moves constantly, and an organisation that can describe a recent change is one that's actually tracking it.

A note on scams

Remote BCBA postings attract fraudulent listings. Treat these as warning signs: any request for payment or equipment purchase, interviews conducted entirely over text or chat, an offer extended without a live conversation, pressure to decide within hours, or vagueness about which states clients are located in. Legitimate employers will always talk to you, and will always tell you where your clients are.

LAST THING

What I'd actually tell a friend

Remote work is not a lesser version of this job. Done well, it reaches families who would otherwise get nothing, and it lets excellent clinicians stay in a field that burns people out with driving.

But it removes the informal scaffolding that holds clinical practice up — the incidental observation, the colleague who notices you're struggling, the ability to physically step in. Good remote organisations rebuild that scaffolding deliberately. Poor ones simply remove it and call the difference efficiency.

These twelve questions are really one question asked twelve ways: *has this organisation rebuilt what remote work takes away, or has it just pocketed the savings?*

You'll usually know by question three.

listening always,

-S ♥

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Stephanie Teixeira, BCBA, IBA. This guide is educational and reflects general practice considerations; it is not legal, billing, or compliance advice, and it does not replace your own verification of licensure and payer requirements, your employer's policies, or direction from a responsible behavior analyst. Regulatory details cited were accurate as of mid-2026 and change frequently — verify current requirements for your state and payers before acting.